

BONE MARROW TRANSPLANT UNIT University Medical Center

June 28, 2023

Dr. Catherine Roberts
Leukemia Service
University Medical Center

RE: Steven Walker (MRN: AML023)
DOB: 02/17/1969
Diagnosis: Relapsed FLT3-ITD+ AML

Dear Dr. Roberts,

Thank you for referring Mr. Walker for urgent allogeneic stem cell transplant evaluation. I saw him in consultation today, and while his case is challenging, we will proceed with transplant planning.

HISTORY: This 54-year-old construction supervisor was diagnosed with intermediate-risk AML (FLT3-ITD positive, NPM1 negative, ratio 0.68) on April 25, 2022. He received induction with 7+3 + midostaurin starting 05/02/22, achieving CR1 by day 30. Consolidation included 4 cycles of HiDAC with midostaurin maintenance.

Unfortunately, he relapsed in early July 2023 with 42% marrow blasts and FLT3-ITD ratio now 1.2 (high allelic burden). He's currently day 21 of gilteritinib salvage with early response (peripheral blasts cleared, awaiting marrow).

CURRENT ASSESSMENT:

Performance Status: ECOG 1

Comorbidities:

- Hypertension (controlled)
- Former smoker (quit 2022)
- BMI 28

Organ Function:

- Cardiac: EF 60%, no CAD
- Pulmonary: FEV1 92% predicted
- Hepatic: Normal LFTs, no hepatitis
- Renal: Cr 1.0, CrCl 95 ml/min

Psychosocial: Married, excellent support, understanding of risks

TRANSPLANT PLANNING:

Donor Search: Initiated urgent search through NMDP. Patient is Caucasian, common haplotype. Three potential 10/10 matched unrelated donors identified, typing confirmation pending. Siblings being typed as backup (has 2 brothers).

Conditioning: Given FLT3-ITD+ disease and young age, recommend myeloablative conditioning with:

- Busulfan (pharmacokinetic dosing) days -7 to -4
- Fludarabine 40mg/m2 days -7 to -4
- Consider adding sorafenib maintenance post-transplant

Timing: Assuming CR2 achieved with gilteritinib, optimal transplant window is within 4-6 weeks of remission. Tentative admission date: 08/15/23.

Risk Assessment:

- TRM risk: ~15-20% at 100 days
- Relapse risk: ~35-40% at 2 years (high given FLT3-ITD with high ratio)
- OS at 2 years: ~45-50%

RECOMMENDATIONS:

1. Continue gilteritinib until transplant admission
2. Restaging marrow in 10 days
3. Complete infectious disease screening
4. Dental clearance needed
5. Fertility preservation discussion (patient declined banking)
6. Insurance authorization in process

We'll coordinate closely with your team. Please send day 28 marrow results when available.

Best regards,

Richard Thompson, MD
Director, Adult BMT Program
Direct line: 555-9876